CARVE-OUT DECISION · SCORECARD

The Carve-Out Decision.

The 10-factor decision framework we walk self-funded clients through to evaluate whether pharmacy carve-out is the better structure than bundled medical-pharmacy administration.

01 Before You Score: Documents to Have Open

The scorecard is a conversation, not a calculator. Pull these reference documents before walking through the ten factors. Anything you cannot locate is itself a finding worth flagging.

- ☐ Current PBM contract and any executed amendments
- ☐ Last 12 months of pharmacy claims summary, with rebate detail
- ☐ Most recent rebate report and pass-through reconciliation
- ☐ Specialty pharmacy spend breakdown by therapeutic class
- ☐ Any market check or RFP within the last 24 months
- ☐ Benefits committee notes on pharmacy oversight

02 Score Your Program

FACTOR	SCORE 1 — FAVOR BUNDLED	SCORE 5 — FAVOR CARVE-OUT	YOUR SCORE (1-5)
Pricing transparency	SCORE 1 Full transparency, competitive pricing.	SCORE 5 Limited visibility, unable to benchmark.	1 2 3 4 5
Rebate flow	SCORE 1 Clear pass-through, auditable.	SCORE 5 Opaque, "competitive" language, no audit rights.	1 2 3 4 5
Data access	SCORE 1 Complete data, flexible reporting.	SCORE 5 Limited data, PBM-controlled reporting.	1 2 3 4 5
Contract flexibility	SCORE 1 Full pharmacy negotiation flexibility.	SCORE 5 Terms tied to medical bundle, limited flexibility.	1 2 3 4 5
Specialty management	SCORE 1 Strong specialty performance.	SCORE 5 Specialty costs growing, limited control.	1 2 3 4 5
Clinical programs	SCORE 1 Documented ROI on clinical programs.	SCORE 5 Programs exist on paper, no outcomes data.	1 2 3 4 5
Renewal position	SCORE 1 Multiple competitive options at renewal.	SCORE 5 Locked in, high switching costs.	1 2 3 4 5

FACTOR	SCORE 1 — FAVOR BUNDLED	SCORE 5 — FAVOR CARVE-OUT	YOUR SCORE (1-5)
Internal capacity	SCORE 1 Dedicated benefits team, broker support.	SCORE 5 Lean team, limited pharmacy expertise.	1 2 3 4 5
Plan size	SCORE 1 5,000+ lives, attractive to PBM market.	SCORE 5 Under 1,000 lives, limited market interest.	1 2 3 4 5
Advisory support	SCORE 1 PBM-specialized consultant on the team.	SCORE 5 Team would benefit from adding pharmacy-specific support.	1 2 3 4 5

Illustrative example for educational purposes. Actual evaluation criteria vary by plan. Higher total scores favor a formal carve-out evaluation; lower totals suggest staying bundled and tightening transparency within the current arrangement.

03 Score Interpretation

Sum your scores across the ten factors for a total between 10 and 50. The score band points to a recommended next step, not a final verdict. Every plan has factors that argue against the trend, and the conversation with your broker is what surfaces them.

TOTAL SCORE 10-20

Stay bundled. Tighten transparency within the current arrangement.

Your current bundled arrangement is performing on the factors that matter most. Focus on closing visibility gaps: pricing benchmarks, rebate reconciliation, and claims data access. Carve-out evaluation is not the highest-leverage move right now.

TOTAL SCORE 21-30

Run a parallel carved-out RFP to compare.

Several factors warrant a deeper look. Request a parallel RFP from at least three carve-out PBMs alongside your current arrangement. The retention offer your incumbent comes back with is itself diagnostic; it tells you what the bundled relationship was actually willing to offer.

TOTAL SCORE 31-40

Strong case for carve-out. Begin formal evaluation.

The factor pattern signals that bundled administration is constraining your pharmacy economics. Build a formal evaluation timeline targeting your next renewal cycle. Engage pharmacy-specialized advisors if internal capacity is thin.

TOTAL SCORE 41-50

Carve-out is likely the better structure. Plan the transition.

The pattern is clear. Begin transition planning now: 6-9 months pre-go-live for a clean carve-out, including PBM selection, contract negotiation, and the implementation realities below. The carve-out decision is the smaller decision; the implementation is what determines whether the value lands.

04 If You Carve Out: Implementation Watch-Outs

The decision gets months of analysis. The transition gets weeks of planning. The first 90 days after go-live determine whether the move delivers what the analysis projected. Four areas where carve-outs most often break:

§ **Member communication starts 60 days pre-transition**
Multiple touches across multiple channels. Members will receive new ID cards, may face network changes, and may need new authorizations on existing therapies. One letter is never enough.

§ **PA transfer is the single most common implementation failure**
Obtain the complete PA inventory from the outgoing PBM. Define a transfer protocol with the incoming PBM. Negotiate a 60-90 day transition override period during which existing authorizations are honored while new ones are processed.

§ **Historical claims data does not auto-follow members**
Require a 24-month historical claims data transfer in the implementation agreement. Specify format, lookback period, and timeline for availability in the new reporting platform. Without it, your new PBM cannot produce year-over-year comparisons or trended utilization analysis.

§ **Pharmacy network differences create access disruption**

Map network differences before go-live, particularly specialty pharmacies managing complex conditions. Identify members using pharmacies not in the new network and develop a member-specific communication plan. Establish a dedicated transition support line for the first 90 days.